

Readiness to Train

Prescriptive Analytics for Optimal Training Intensity

KU Leuven & OH Leuven
Project Overview & Problem Statement

1. Core Objective

The goal of this project is to **optimise the performance of players on match day** by developing a prescriptive analytics system that estimates individual player **Readiness to Train**. Using **Causal Machine Learning** within a **Dynamic Treatment Regime (DTR)** framework, the system will recommend an optimal daily training intensity for each player — delivered as a continuous score between 0 and 1. A score of 0 indicates the player should rest; a score of 1 indicates the player can train at maximal intensity.

The system will be fully **individualised**: each player will receive a personalised score derived from their own physiological profile, training history, and evolving daily state. The ultimate objective is to find the training intensity sequence across the pre-match cycle that maximises each player's physical output on match day.

2. Partnership & Data

The project is a collaboration between **KU Leuven** (methodology, analysis) and **OH Leuven** (professional football club, data provider). OH Leuven operates a systematic daily player monitoring programme covering GPS load metrics, subjective wellness questionnaires, medical status, and match performance. Four raw datasets are available:

Dataset	Rows	Cols	Players	Start	End	Days
Readiness_Data	14,359	24	28	2024-07-01	2026-02-17	597
Raw_Data	9,968	39	84	2024-05-02	2026-03-01	670
Sessions	1,206	9	—	2024-05-02	2026-03-01	670
Games	403	9	24	2025-07-27	2026-02-28	216

The **processed dataset** (RTT.xlsx) merges all four sources into a single analysis-ready file with 46 engineered features per player-day. Readiness_Data is the base; Raw_Data provides detailed GPS/HR at session level (shifted +1 day so it appears as "yesterday" data); Games provides match performance data (also shifted +1 day). Player overlap: all 28 Readiness_Data players appear in Raw_Data; 23 of 28 appear in Games.

Data dictionaries: Two auto-generated PDF documents describe every variable in detail. The **Raw Data Dictionary** (data/raw/Raw Data Dictionary.pdf) documents all columns across the four raw datasets. The **RTT Data Dictionary** (data/processed/RTT Data Dictionary.pdf) documents all 46 columns in the processed dataset, including engineered features and their temporal positions.

Temporal ordering within each row: Variables in a single row have different temporal positions. "Yesterday" columns contain data from day $t-1$ (fully known). Morning wellness z-scores are measured at the start of day t

(before any activity decisions). Post-assessment variables (Activity Type Today, Selected) are determined after the morning assessment and represent the treatment decision — they must never be used as predictors in standard ML.

3. Sports Science Foundation

Training stress produces fatigue, which during recovery is followed by adaptation (supercompensation). The ACWR (Acute:Chronic Workload Ratio) captures where a player sits on this curve: the sweet spot is 0.8–1.3; values above 1.5 are flagged as danger zones (Gabbett, 2016). GPS metrics are individualised as percentages of each player's match benchmarks. The training intensity score $A(t)$ is the tanh-compressed harmonic mean of these GPS percentages, soft-capped in $[0, 1]$. The typical weekly structure (5–6 training days followed by a match) creates player-specific match cycles central to the DTR formulation.

4. Why This Is a Causal Problem

Standard predictive modelling asks *"given the data, what will happen?"*. This project asks *"given a player's full history up to today, what training intensity today best contributes to future match-day performance?"*. This requires causal reasoning for three reasons:

- 1. Time-Varying Confounding Affected by Prior Treatment.** Covariates at time t (fatigue, readiness, ACWR) are simultaneously *consequences* of treatment at $t-1$ and *causes* of treatment at t . Classical regression cannot handle this correctly — this is precisely the setting for which G-methods were developed.
- 2. Treatment-Confounder Feedback.** Coaching decisions are observational. Hard sessions are prescribed when players appear fresh; recovery is prescribed when fatigue is high. The observed training-performance association conflates physiological response with coach selection behaviour.
- 3. Sequential Decision-Making.** The treatment is a repeated, time-varying intervention. The question is not just "what training today?" but "what *sequence* of training intensities over the cycle maximises match-day performance?" — a DTR problem.

5. Causal Framework

Symbol	Role	Description
$L(t)$	Covariates	Player state at time t : wellness, ACWR, GPS load, schedule position
$A(t)$	Treatment	Training intensity $[0,1]$: $\tanh(\text{harmonic mean of GPS benchmark } \%)$
Y	Outcome	Match-day performance: high-intensity metrics per ball-in-play minute

The problem is modelled as a **longitudinal causal DAG** (Directed Acyclic Graph) with the following structure within each match cycle:

- At each time step t , the player's **covariates $L(t)$** (morning wellness, fatigue, ACWR) causally affect the coaching staff's **treatment decision $A(t)$** (what training intensity is prescribed that afternoon).
- The treatment $A(t)$ then causally affects the player's **state $L(t+1)$** the next morning (training changes fatigue, soreness, adaptation level).
- This creates a **feedback loop**: $L(t) \rightarrow A(t) \rightarrow L(t+1) \rightarrow A(t+1) \rightarrow \dots$ repeating across the match cycle until match day, when the final player state and cumulative training determine the **match outcome Y** .
- Between cycles, the match outcome Y feeds back into the first state of the next cycle (match fatigue carries over).

The figure below shows an example DAG for Player 1, match cycle 4 (schematic view with grouped covariates):

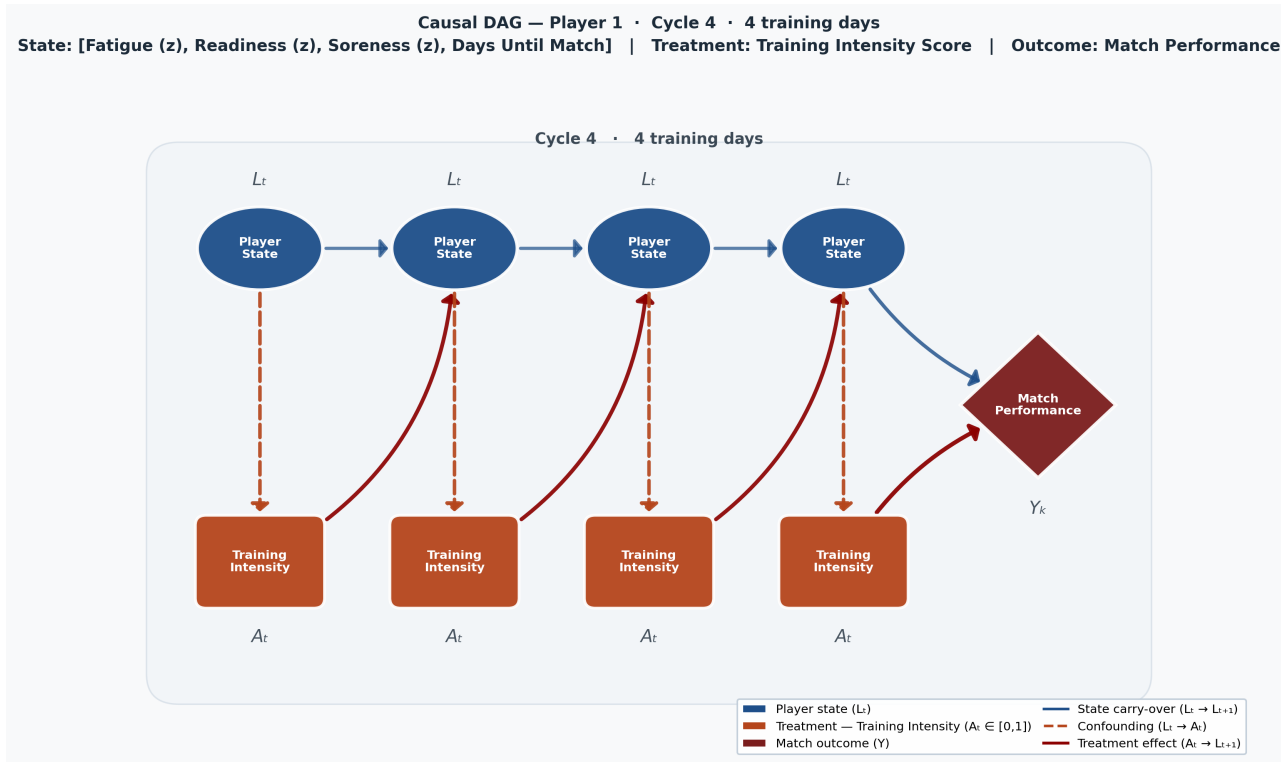


Figure: Causal DAG for Player 1, match cycle 4 (schematic). Blue ellipses = player state $L(t)$, amber rectangles = treatment $A(t)$, crimson diamond = match outcome Y .

This DAG structure is generated automatically for each player and each match cycle using the DAGCreator class (499 DAGs total, stored in *images/DAGs/player X*). It encodes the causal assumptions needed for G-methods: confounding edges ($L \rightarrow A$), treatment effects ($A \rightarrow L$ at $t+1$), state carryover ($L \rightarrow L$ at $t+1$), and inter-cycle feedback ($Y \rightarrow L$ at cycle $k+1$).

6. Key Challenges

Small N, Large T

Only 28 players are monitored in Readiness_Data, but each player has up to 597 days of observations. The signal is likely dominated by **within-player dynamics**. Models must be parsimonious or leverage partial pooling to avoid overfitting across players. Methods must balance individual-level tailoring with the limited between-player sample size.

Time-Varying Confounding Affected by Prior Treatment

Covariates $L(t)$ at time t are simultaneously consequences of treatment at $t-1$ and causes of treatment at t . This is the defining feature of the problem and the reason standard regression fails. The appropriate causal identification framework is the theory of G-methods (Robins, 1986): G-computation, Marginal Structural Models with IPTW, or G-Estimation / Structural Nested Mean Models.

Observational Data with Coach Selection Bias

Training loads are prescribed by coaching staff based on observed player state. Players who appear fresh receive harder sessions; fatigued players receive recovery sessions. This creates a systematic association between baseline health and training dose that confounds any naive regression of load on performance.

Sparse Match-Day Outcomes

The outcome Y (match performance) is observed only on match days. With a 1-week cycle, each player contributes approximately one outcome observation per 6 rows of covariate/treatment data. The Games dataset covers 24 players over 27 match dates (403 player-match rows). Mean minutes played: 67.4 min/match; mean high-intensity distance per BIP: 18.75 m/min.

